
ADHD.ME

found earlier.
treated closer to home.
counted properly.

Aboriginal and Torres Strait Islander children carry more attention and hyperactivity difficulty than other Australian children, and receive less treatment for it. This is a proposal about closing that particular gap, and what closing it would prevent.

We acknowledge the Ngunnawal people as Traditional Custodians of the ACT and region, and recognise other people and families with connection to these lands — including those identifying as Ngambri (Kamberri). We pay respect to Elders past, present and emerging.

more need. less treatment.

15.8%

of Aboriginal children with clinically significant hyperactivity — against 9.7% of other children

two-thirds

less likely to receive stimulant treatment, where both parents are Aboriginal

30%

more likely than other children to be living with disability

none

validated ADHD symptom norms exist for most Aboriginal and Torres Strait Islander groups

Western Australian Aboriginal Child Health Survey; linked-data cohort study of stimulant prescribing in Western Australia (BMC Pharmacology and Toxicology, 2015); Australian Evidence-Based Clinical Practice Guideline for ADHD (AADPA, 2022), section on Aboriginal and Torres Strait Islander peoples.

none of these are about the child.

THE INSTRUMENTS

There are no psychometric studies of ADHD questionnaires, and no symptom norms, for most Aboriginal and Torres Strait Islander groups. The tools were normed on somebody else.

THE HISTORY

Fear tied to eugenics and the Stolen Generations — which the guideline notes is still in living memory.

THE RECEPTION

Discrimination, racism and ignorance when seeking mental health support, which suppresses help-seeking before a clinician is ever involved.

THE RECORD

Under-identification of Aboriginal and Torres Strait Islander status in health settings, so the list of children to look at never forms.

THE LABEL

A cultural dislike of labelling and of diagnostic stigma, and reluctance to seek help until difficulties are extreme.

Barriers as set out in the Australian Evidence-Based Clinical Practice Guideline for ADHD (AADPA, 2022). Every one of them is a property of the system doing the assessing, not of the person being assessed.

from the classroom to the cell.

IN SCHOOL

8.6% → 25%

share of enrolments, against share of students suspended

BY YEAR 7

98%

increase in disciplinary absences between Year 6 and Year 7

IN DETENTION

89%

of children assessed had a severe neurodevelopmental impairment, most never diagnosed

IN PRISON

31% v 10%

adult ADHD among Aboriginal prisoners, against non-Aboriginal prisoners

The behaviour a child is suspended for — disruption, disengagement — is the diagnostic criteria for the condition nobody assessed.

School data: national and Queensland suspension reporting, 2021–2023. Detention: Bower et al., BMJ Open 2018 (Banksia Hill, WA). Prison: adult ADHD identified in NSW imprisoned populations, cited in the AADPA guideline. Different jurisdictions and years; each figure is indicative of a stage, not a single cohort followed through.

the justice targets are going backwards.



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Closing the Gap targets on track, halfway to the 2031 deadline



3% → 37%

share of the adult population, against share of people in custody



2,500

Aboriginal and Torres Strait Islander adults imprisoned per 100,000 — up from 1,925 in 2019



No change

Target 11, youth detention, against a 30% reduction by 2031

Productivity Commission Closing the Gap dashboard and Annual Data Compilation (July 2026); ABS Prisoners in Australia (2025) and Corrective Services (March quarter 2026). Target 10 seeks a 15% reduction in adult imprisonment by 2031; Target 11 a 30% reduction in youth detention.

32% fewer convictions. the same people.

A Swedish national cohort of 25,656 people with ADHD, followed from 2006 to 2009. Criminality fell 32% in men and 41% in women during periods on medication.

The comparison is within-individual.

Not treated people against untreated people, where the two groups differ in a hundred ways. The same person, on medication and off it. That design is why the finding survives.

Lichtenstein et al., "Medication for Attention-Deficit-Hyperactivity Disorder and Criminality", New England Journal of Medicine, 22 November 2012. No equivalent Australian study exists, and none specific to Aboriginal and Torres Strait Islander people.

ADHD does not cause incarceration.

WHAT DRIVES IT

Colonisation and dispossession, poverty, housing, over-policing, bail laws, and the age of criminal responsibility. Nothing in this proposal touches any of them.

WHAT IS ALSO TRUE

Among children already in contact with the system, severe neurodevelopmental impairment is close to universal, and mostly undiagnosed until a court asks.

WHAT WE PROPOSE

Treating a treatable condition in children who are not currently offered treatment. That is health care, not a theory of crime.

A health intervention cannot fix a justice problem whose causes are structural. What it can do is stop the health system contributing to one, by treating a condition it currently leaves undiagnosed until a court is involved.

THE MODEL

say it in your words. book with one person.

ADHD.ME is a patient-facing finder and a practice-facing console on one engine. It never assesses anybody. It routes a person to a clinician who can, and then measures whether they got there.

- ✓ Say what you need — in your own words, including how you want to be treated.
- ✓ See who is near you — by suburb, language, care area, and how that clinician works.
- ✓ Book once — assessment, baseline and follow-up with the same person.

ADHD assessment
*that takes
you seriously.*

Describe the GP you're looking for...



[Try a demo scenario >](#)

the product is that list, inverted.

THE INSTRUMENTS

ADHD.ME never screens and never scores. It carries no questionnaire, so it cannot inherit a questionnaire normed on the wrong population. It routes to a clinician who assesses.

THE RECEPTION

A person says what they want from care — including not being judged — and is matched on those clinician attributes, by suburb and language. Matching is on the clinician, never on the patient’s symptoms.

THE RECORD

The register is built inside the service, from its own records, under its own rules. Identification improves without data leaving the building.

THE LABEL

Scheduling language only. No diagnosis talk, no urgency, no clinical claims on any patient-facing surface — enforced by lintars that fail the build, not by a style guide.

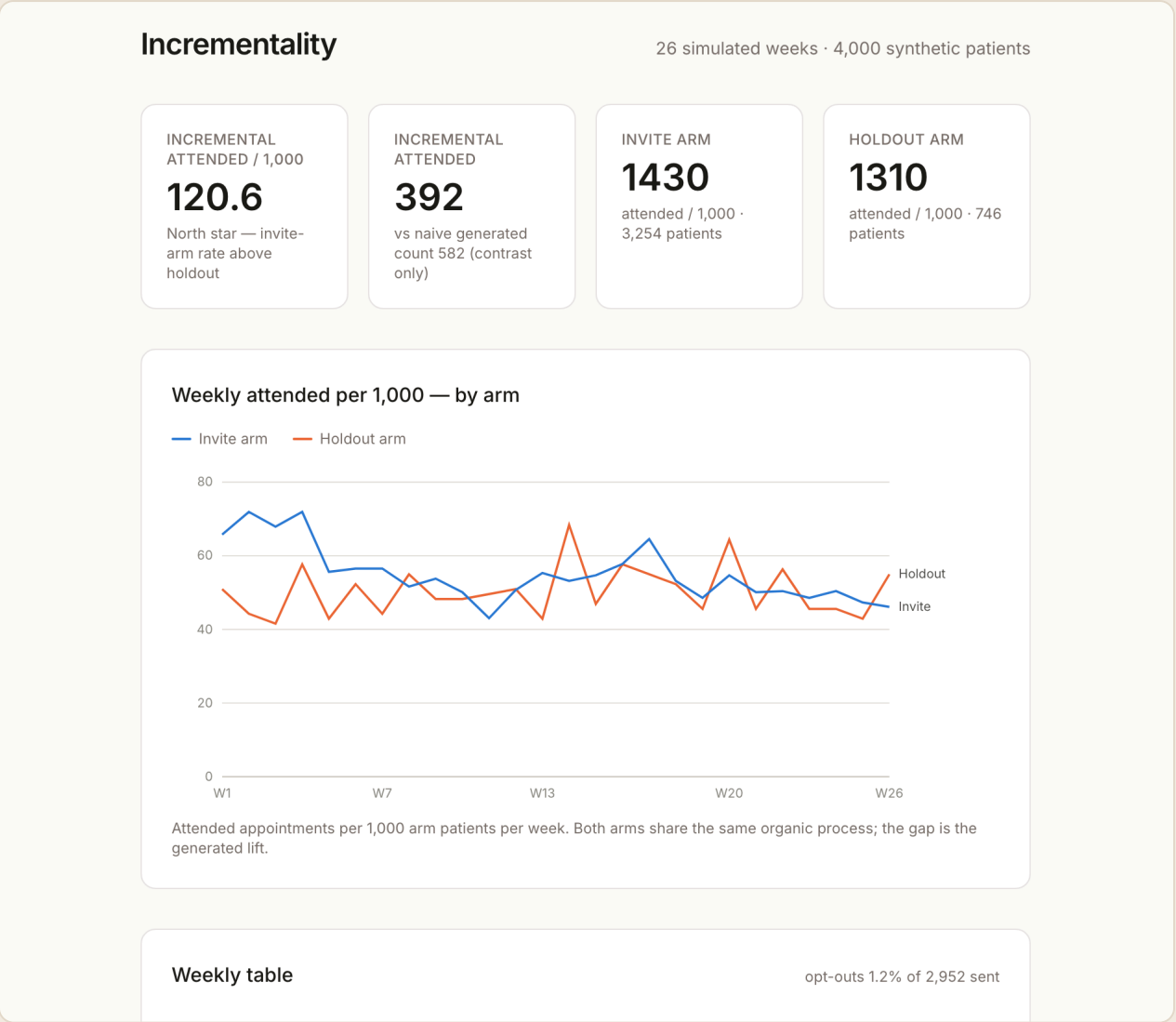
THE EVIDENCE GAP

Every invitation runs against a randomised holdout, so the pilot produces the Australian evidence the guideline says does not exist.

a program that can report zero.

A randomised holdout arm runs continuously beside the invited arm. The reported figure is the difference between them — never the number of people the program touched. If it did nothing, the number is zero and the report says zero.

The 2024 Review of the National Agreement found that governments cannot demonstrate what is working. This is a small, literal answer to that finding.



Synthetic practice engine — 26 simulated weeks, 4,000 synthetic patients. No real patient data exists in this build, and none will without the partner organisation’s decision.

WHOSE INFRASTRUCTURE THIS IS

built to be handed over.

PRIORITY REFORM TWO

Deployed as the partner organisation's own infrastructure. We never hold the patient relationship, the clinical record, or the Medicare claim.

PRIORITY REFORM FOUR

The partner organisation is the data custodian. Nothing is disclosed, published or shared without its decision.

PRIORITY REFORM ONE

The partner sets eligibility, message content, and whether the pilot proceeds at all — written into the agreement rather than left to goodwill.

NO PARTNER YET

We have no Aboriginal or Torres Strait Islander partner organisation today. Securing one is Phase 0, before any build.

one year of one cell, or nine hundred assessments.

\$3,600

one child, one day, in youth detention

\$1.3m

one child, one year

~900

ADHD assessments the same year
would fund

\$159,510

one adult, one year, in prison

Detention is the most expensive thing this system does, and the least effective. Assessment is among the cheapest.

Report on Government Services 2026 (youth justice and corrective services). Assessment costed at the private-market rate; delivered through a community-controlled service the per-assessment cost is lower, and the ratio improves.

one region, twelve months, one auditable number.

PHASE 0 · MONTHS 1–3

Partnership first. No build, no data and no deployment until an Aboriginal Community Controlled Health Organisation has agreed scope, governance and data terms in writing.

PHASE 1 · MONTHS 4–9

Identification and assessment pathway inside partner services, under the partner’s rules, with the randomised holdout running from the first day.

PHASE 2 · MONTHS 10–12

Read-out: assessments completed, treatment initiated, treatment retained at three months — and a plain account of what did not work.

PUBLICATION

Published whether positive or null, with the partner as co-author, and the underlying data remaining theirs.

Twelve months cannot measure incarceration, and this proposal does not pretend otherwise. The pilot measures the health steps the evidence links to it — assessment completed, treatment initiated, treatment retained — and reports those. Deliberately unpriced until co-designed in Phase 0.

a finding, not a licence.

THE DELIVERABLE

An audited incrementality read-out on one Closing the Gap-relevant measure, published whether it is positive or null. Not a portal subscription, and not a dashboard nobody opens.

THE COMMERCIAL MODEL

A flat platform fee per service. No percentage of clinical revenue and no per-referral payment, ever — a compliance posture under fee-splitting and inducement rules, not a pricing preference.

INDICATIVE SCALE

\$4,800 to \$18,000 per service per year at steady state, by size; a twelve-week pilot at \$2,400 per service. The program figure is set with the partner in Phase 0, not before it.

AT THE END OF IT

The register and the infrastructure stay with the partner organisation. No lock-in, no dependency, and nothing that has to be renewed for the community to keep the capability.

Governments buy certainty. What is on offer is a number allowed to come back negative, and a capability that stays behind either way.

three of us. the fourth seat is not ours to fill.



Vikram Ganeshalingam

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Vacant, *deliberately*

ABORIGINAL AND TORRES STRAIT ISLANDER
GOVERNANCE

A condition of this work proceeding — not
an advisory seat added after funding.

four things, in this order.

-  An introduction to Winnunga Nimmityjah, the ACT's Aboriginal Community Controlled Health Service, and to NACCHO — so partnership begins with the sector rather than with us.
-  Scoped pilot funding, released in two tranches — partnership first, build second.
-  Access to linked health, education and justice data at regional level, on Priority Reform Four terms set by the partner.
-  A named contact in Health and one in the Attorney-General's portfolio, because this sits across both.

If the holdout shows no effect, that is the finding, and we will publish it.

thank you

Vikram Ganeshalingam

Dr Anubhav Saxena

Stefan Thottunkal

We acknowledge the Ngunnawal people as Traditional Custodians of the ACT and region, and recognise other people and families with connection to these lands, including those identifying as Ngambri (Kamberri). We pay respect to Elders past, present and emerging, and to the many First Nations across this country. Any work described here proceeds only with, and under the authority of, the communities it concerns.

add contact email before sending